

NHS Devon Integrated Care Board Service Specification

Service Specification No. :	1
Service:	Administration of Gonadorelin Analogues - Triptorelin -Decapeptyl® and Goserelin -Zoladex® For patients with Carcinoma (Ca) of the Prostate
Commissioner Lead:	Paul Green, Director for Primary Care
Provider Lead:	
Period:	1 st July 2017 – 31 st March 2029 (3 months' notice will be required to terminate this contract by either party)
Date of Review:	Annual review re inflation Formal LES review: October 2028

1. Population Needs

1.1 National/local context and evidence base

Gonadorelin analogues are used primarily, though not exclusively, in the treatment of carcinoma of the prostate. There are a number of treatment regimes, which vary in the detail of their programme of administration and main purpose. Broadly they can be divided on the basis of the progress of the disease into advanced local disease and metastatic disease. The central usage, however, remains the treatment of metastatic cancer of the prostate. Currently it is estimated that over 95% of the scripts for gonadorelin analogues are written for Ca prostate.

Virtually all the prescriptions issued for injectable Gonadorelin analogues are written by GPs and most of these are also administered by GPs. In some practices an appropriately trained practice nurse will site the depot implants or injection.

This service is intended for the administration of Gonadorelin analogues for patients with Ca Prostate.

2. Outcomes

NHS Outcomes Framework domains & Indicators

<https://www.wp.dh.gov.uk/publications/files/2012/11/121109-NHS-Outcomes-Framework-2013-14.pdf>

Domain 1	Preventing People from dying prematurely	✓
Domain 2	Enhancing quality of life for people with long term conditions	✓
Domain 3	Helping people to recover from episodes of ill health or following injury	✓
Domain 4	Ensuring people have a positive experience of care	✓
Domain 5	Treating and caring for people in a safe environment and protecting them from avoidable harm	✓

3. Scope

3.1 Aims and objectives of service

Patients with an established diagnosis and agreed treatment plan of Carcinoma of the Prostate can undergo part of their treatment safely, effectively and conveniently close to their home in accordance with the locally agreed

prescribing Guidelines.

Please note Section 4.2 below.

3.2 Service description/care pathway

This service is delivered under an agreed prescribing guideline approved by the local Formulary. The Provider is required to:

- **Provide a register** – The provider will need to produce and maintain a valid up-to-date register of patients being treated as part of this enhanced service.
- **Demonstrate a call and recall system** - The provider will need to ensure a systematic call and recall of patients on this register is taking place, and have in place the means to identify and follow up patients in default.
- **Agree a joint clinical management programme** - Patients should be managed on the basis of individual treatment plans which will normally be drawn up by local consultants. The provider will be expected to follow these treatment plans unless there has been discussion and agreement with local consultants to modify them. The plan should give the reason for treatment, the agreed treatment programme and the planned duration. This plan should be consistent with any agreed prescribing guideline.
- **Support the education of both newly diagnosed patients and those with established disease.** The secondary care oncology team will provide the main source of advice for both newly diagnosed patients and those with established disease. The provider will reinforce and supplement that advice where appropriate to do so.
- **Record keeping** - To maintain adequate records of the service provided, incorporating all known information relating to any significant events e.g., adverse reactions, hospital admissions, and relevant deaths of which the practice has been notified.
- **Ensure primary care staff training** – The provider must ensure that all staff involved in providing any aspect of care under this scheme have the necessary training and skills to do so. Practices should be able to demonstrate that they have in place a policy to cover staff training and maintenance of skills.
- **Provide safe and suitable facilities for undertaking invasive procedures** – The ICB should be satisfied that the provider undertaking the “Administration of Gonadorelin Analogues for Patients with Ca of the Prostate Service” have adequate and appropriate facilities and equipment comparable to those required for the safe provision of any invasive procedure.

Providers will be required to record activity for this service with the following READ Code: Insertion of Gonadorelin analogue Implant 7G2AC.

3.3 Population covered

The registered practice population

3.4 Any acceptance and exclusion criteria

None

3.5 Interdependencies with other services

The provider will be expected to develop effective links with Secondary Care Urology Consultants. Responsibility is shared:

Responsibility of Secondary Care:

- Diagnosis of condition and ensuring other treatment options have been fully explored
- **Initiation of treatment.** The first dose of Gonadorelin, and any required course of antiandrogens (usually 2-3 weeks) are prescribed and given in secondary care

- Monitoring for response and adverse drug reactions (ADRs) during initiation period
- Liaison with the provider to share the patient's care after the first dose has been given. The default duration of Gonadorelin treatment is life-long unless specified otherwise. For the small group of patients undergoing intermittent androgen deprivation therapy, the same principle applies, i.e. initiation is carried out in secondary care and maintenance in primary care; and the provider to be notified of the start and stopping schedule of treatment
- Prostate specific antigen (PSA) should be measured as recommended during follow up, using PSA tracker
- Arrange follow up schedule and monitoring of patient's response and inform GP of results and any change in condition or treatment
- Responding to issues raised by GP after care of patient has been shared
- Advising the provider on related issues such as drug interactions etc.

Responsibility of The Provider:

- Confirm or decline request to share patient's care as soon as possible. Monitoring the patient's overall health and well-being and observing patient for evidence of ADRs/abnormalities and raising with secondary care clinician if necessary
- Prescription of drug
- Arranging for regular administration
- Ensuring advice is sought from the secondary care clinician if there is any significant change in the patient's physical health status
- Stopping treatment in line with secondary care clinician's request

4. Applicable Service Standards

4.1 Please use the online version of the local formulary for guidance on the appropriate administration of Gonadorelin Analogues (Triptorelin - Decapeptyl® and Goserelin - Zoladex®) for patients with Carcinoma of the Prostate

<https://southwest.devonformularyguidance.nhs.uk/formulary/chapters/8.-malignant-disease/8-3-sex-hormones-and-hormone-antagonists/8-3-4-hormone-antagonists>

NB Please note that Leuprorelin Acetate, Prostav® which was previously included in this specification is no longer included in the South Devon Formulary.

4.2 Consultants will be encouraged to prescribe according to the formulary, however the formulary contains the following statement:

"Given the emotive clinical indications this therapy is used to treat, there is no compulsion at present to change existing patients' therapy or agreed treatment regimens. Local specialists agree that all GnRH analogues are equally effective and will continue to recommend treatment with a 'GnRH analogue' rather than a specific brand of product to be used, unless clinically indicated. Specialists agree that general practitioners can change the product prescribed within this class of therapy to complete courses of therapy for existing patients as clinically appropriate and after informed discussions with the patient".

This therefore allows for a degree of flexibility so that a practice will be eligible for payment under this specification where a non-formulary product is used because the patient has been initiated on that product elsewhere, in secondary care, or prior to the start of this specification, and the patient is stable on that product.

The prescriber of the medication legally assumes clinical responsibility for the drug and the consequences of its use. The prescriber should:

- keep informed about the medicines that are prescribed for the patient
- be able to recognise serious and frequently occurring adverse side effects
- make sure appropriate clinical monitoring arrangements are in place and that the patient and healthcare professionals involved understand them
- keep up to date with relevant guidance on the use of the medicines and on the management of the patient's condition

5. Key Service Outcomes

Patients will receive care in a primary care setting, reducing the number of patient contacts with secondary care and the number of journeys to hospital.

6. Applicable Quality Requirements

Further guidance on the diagnosis and treatment of prostate cancer can be found at <https://www.nice.org.uk/guidance/cg175>

Audit and Incident requirements are described below.

Formal incidents should continue to be reported to NHS England using the SEA form.
<https://www.england.nhs.uk/south/wp-content/uploads/sites/6/2015/08/sea-incident-report-form.doc>

For any other issues of poor quality or service, parts of the system not working well or negative patient experience. Please continue to complete a Yellow Card form which can be found on the ICB'S website:
<https://onedevon.org.uk/contact-us/pitch-yellow-card/>

7. Financial Requirements

All patients within the scheme will attract an annual fee of £62.22

DDRB will be used as the start point for annual 'inflationary' review, with any deviation from DDRB uplift requiring clear rationale.

In the event of a pandemic or other 'level 4 health' event, commissioner and LMC will meet urgently to review this specification giving consideration to:

- suspension of some or all elements of it,
- redirection of patients to alternative provider(s),
- in the event of actioning either a) or b) determining appropriate income protection arrangements for GP provider

8. Information Requirements

The provider shall submit every six months the number of patients to whom the service is provided.

9. Location of Provider Premises

The provider premises.